FORGE

PHASE 1 — FOUNDATION

Week 4 of 36

Trauma Awareness

Sessions 10–12

Week 4: Trauma Awareness

Facilitator Notes for Week 4

Week 4 is the most emotionally intense week in Phase 1, and its message has to land both ways: trauma is not an excuse for harmful behavior; it is an explanation that creates a pathway to change. Session 10 defines trauma with SAMHSA's Three E's and has each man privately count his ACE score. Session 11 shows how fight, flight, freeze and fawn play out in the dorm, and why a small moment can set off a reaction that belongs to the past. Session 12 turns that understanding into action: safe coping, and a Safe Coping Card each man keeps within reach. The tools from Weeks 2 and 3 come back here as the gap between reacting and responding. Week 5 then turns outward, to listening and communication.

This week also draws the line every FORGE mentor will live by: trauma-aware, not therapeutic. When a mentee says something that needs professional help, the job is to listen, validate and refer, and that is your job this week too. Confirm your mental health referral contact before Session 10, never ask a man for his score or his story, and expect the material to stir your own history as well.

What to Watch For

Trauma activation: This is the most emotionally intense week in the entire Phase 1 curriculum. Specific risks:

- The ACE activity in Session 10 may be the first time someone has ever counted how many adverse experiences they had. A high score can be validating (explaining a lifetime of struggle) or devastating (confronting the full scope of what happened). Watch faces during the activity and during the hand-raise.
- Session 11 (trauma responses) may cause participants to relive specific moments, especially when connecting past events to current reactions. Watch for dissociation — a blank stare, loss of eye contact, seeming "checked out." If you see this, gently ground the person: "Hey, [name]. Come back to the room. You're here. You're safe. Take a breath."
- Session 12 is designed to be stabilizing — it moves from understanding to action. But some participants may still be processing Sessions 10 and 11. Be patient.

The participant who can't stop talking about their trauma: Once the door opens, some men will want to share everything — in detail, repeatedly, to anyone who will listen. This is understandable but requires management. Redirect gently: "I hear you. That's important. And right now I want to make sure everyone in the group has space. Let's talk more after the session." Then follow through — actually have that conversation.

Copyright © 2026 Positive Outreach Development Society, Inc.

The participant who shuts down completely: Some men will go silent after the ACE activity or the trauma response discussion. They might seem fine but are actually in freeze. Don't force participation. A quiet check-in after the session — "That was heavy material. How are you sitting with it?" — can make the difference between a man who processes and a man who walls off.

Peer support emerging: Watch for men checking on each other between sessions. This is the cohort container working. Acknowledge it when you see it: "I noticed you checking in on [name] after last session. That's exactly what Service Over Self looks like."

Common Week 4 Challenges

"You're making excuses for criminals."

> *This can come from participants themselves. Address it head-on: "Nobody in this room is making excuses. Understanding why you did what you did is not the same as excusing it. You can hold two truths at the same time: 'I experienced real harm as a child' AND 'I caused real harm as an adult.' Both are true. Neither cancels the other. FORGE asks you to be honest about both."*

"I don't have any trauma — I had a normal childhood."

> *"Maybe you did. And if so, your ACE score will be low and this week will help you understand other people better. But sometimes what we call 'normal' is just what we got used to. A kid who watched his parents fight every night might call that normal. A kid who got hit with a belt for talking back might call that discipline. Whether you experienced trauma is for you to determine — not me. Just be honest with yourself."*

"So now what? I know I have trauma — what do I do about it?"

> *"That's the right question. And the answer has two parts. Part one: keep using the tools you're learning here — understanding your thinking, regulating your emotions, recognizing your responses, and using safe coping. These tools directly address the effects of trauma. Part two: if your trauma is severe — if you're having nightmares, flashbacks, panic attacks, or symptoms that won't quit — consider talking to mental health services. There's no shame in it. Just like a broken bone needs a doctor, a deeply wounded mind needs professional care. FORGE teaches you to understand trauma. A professional can help you heal it."*

"I don't trust mental health staff here."

Copyright © 2026 Positive Outreach Development Society, Inc.

> *This is often legitimate. Many participants have had negative experiences with institutional mental health services. Acknowledge it: "I hear you. Not every mental health worker is good at their job, and not every experience with them has been helpful. But the option is there, and if you find the right person, it can make a real difference. In the meantime, everything you're learning in FORGE is building your capacity to manage what you're carrying. You're not alone in this."*

A participant who is activated beyond what the group can hold:

> *If someone is in genuine crisis — severe emotional distress, dissociative episode, disclosure of active self-harm or suicidal ideation — act immediately. Stay calm. Stay with the person. Ask the Senior Mentor or a trusted participant to continue the session. Take the individual aside. Use grounding techniques. Assess safety. Contact mental health services if needed. After the crisis is managed, follow up with the individual within 24 hours and continue checking in.*

A Note on Facilitator Self-Care

This week will affect you too. If you have your own trauma history — and most facilitators who do this work do — the material may activate your own responses. That doesn't make you unfit to facilitate. It makes you human. But you need to take care of yourself:

- Process the sessions with a co-facilitator, supervisor, or trusted peer after each one
- Use the same regulation techniques you're teaching
- Know your own limits — if you're too activated to hold space, it's okay to lean on the Senior Mentor or ask for support
- Do not carry the weight of every participant's story by yourself. You cannot pour from an empty cup

Preparation for Week 4

- Review SAMHSA's Three E's framework (Event, Experience, Effect) — be able to explain it clearly and simply
- Prepare ACE (Adverse Childhood Experiences) questionnaire for anonymous scoring activity — enough copies for each participant
- Research current statistics on trauma prevalence in incarcerated populations
- Review fight/flight/freeze/fawn responses and prepare dorm-specific examples for each
- Prepare the safe vs. unsafe coping strategies list
- Prepare blank "personal safety plan" or "safe coping card" templates
- **Critical:** Be prepared for the heaviest emotional content in Phase 1. Week 4 discusses childhood trauma, abuse, neglect, and ACEs. Some participants may be hearing the word "trauma" applied to their own experiences for the first time. Have your mental health referral contact confirmed and available.

Copyright © 2026 Positive Outreach Development Society, Inc.

- Consider whether to invite a mental health professional to be present or on standby during Session 10 (ACE activity)
- Remind yourself: FORGE is trauma-aware, not therapeutic. Your job is to help men understand trauma, not to treat it. Know the line and hold it.

Week 4 Overview

Purpose: Help participants understand trauma — what it is, how it works, and how it shapes behavior — without turning FORGE into therapy. By the end of this week, every participant should understand the basics of trauma science, recognize trauma responses in themselves and others, distinguish between safe and unsafe coping strategies, and have a personal safety plan they can actually use. The key message: trauma is not an excuse for harmful behavior. It is an explanation that creates a pathway to change.

Sessions This Week:

- Session 10: Understanding Trauma
- Session 11: Trauma Responses in the Dorm
- Session 12: Coping and Safety

Materials Needed:

- Journals/notebooks
- Pens/pencils
- ACE (Adverse Childhood Experiences) questionnaire — anonymous scoring copies (1 per participant)
- SAMHSA Three E's handout (1 per participant — simple one-page summary)
- Trauma responses reference sheet (fight/flight/freeze/fawn with correctional examples)
- Safe vs. Unsafe Coping Strategies handout (1 per participant)
- Blank "Safe Coping Card" templates (1 per participant — index card size or similar)
- Talking piece for circle process
- Easel paper or whiteboard (if available)
- **Confirm mental health referral contact and availability for this week**

Important facilitator note for the entire week: This is the most emotionally intense week in Phase 1. Trauma content can activate participants in ways that are unpredictable. Before starting this week, read the facilitator notes above. Know your referral resources. Be prepared to hold space without trying to fix.

Copyright © 2026 Positive Outreach Development Society, Inc.

SESSION 10: Understanding Trauma

Duration: 2 hours
Facilitator(s): Program Lead + Senior Mentor (if available)

Learning Objectives

By the end of this session, participants will be able to:

1. Define trauma using SAMHSA's Three E's framework (Event, Experience, Effect)
2. Explain what ACEs are and how they relate to later life outcomes
3. State the prevalence of trauma among incarcerated populations
4. Distinguish between "trauma as excuse" and "trauma as explanation"
5. Complete an anonymous ACE score activity and reflect on its personal meaning

Session 10 Checklist

- ☐ Elevator speech said together, then by 2–3 men alone
- ☐ Room set up in circle
- ☐ ACE questionnaires ready (anonymous copies)
- ☐ SAMHSA Three E's handouts distributed
- ☐ Mental health referral contact confirmed and available
- ☐ Opening circle completed — tone set for the week with safety language
- ☐ Emotion tracking patterns reviewed
- ☐ SAMHSA's Three E's taught (Event, Experience, Effect)
- ☐ ACEs explained — 10 categories and research findings
- ☐ Trauma prevalence in incarcerated populations shared
- ☐ "Trauma is not an excuse — it is an explanation" clearly delivered
- ☐ Anonymous ACE score activity completed
- ☐ Group hand-raise conducted with dignity
- ☐ Journal reflection completed (10 min writing)
- ☐ Debrief discussion held
- ☐ Closing round completed (one word)

Copyright © 2026 Positive Outreach Development Society, Inc.

- ☐ Homework assigned (journal continuation + emotion tracking + self-care)
- ☐ Safety check: no participant left in visible distress

Session Plan

Elevator Speech (2 minutes)

Start the session with the elevator speech, before anything else. The class says it together, out loud. Then ask two or three men to say it alone, without looking. Call on different men each session, so every man has said it alone before Session 18.

Opening Circle (10 minutes)

> **Facilitator:**
>
> *"Welcome back. Before we check in, I want to set the tone for this entire week.*
>
> *This week is called Trauma Awareness. We're going to talk about things that some of you have never talked about before — and some of you have talked about but never understood. Childhood experiences. Adversity. Pain that shaped who you became.*
>
> *Two things are true at the same time:*
>
> 1. *This material is important enough that I won't water it down.*
> 2. *Your safety in this room matters more than any lesson plan.*
>
> *If at any point this week you need to step outside for a minute, you can. No explanation needed. Just step out, take a breath, and come back when you're ready. If something comes up that you need to process beyond what this group can hold, I will help you connect with someone who can support you.*
>
> *This is FORGE — not therapy. We're going to understand trauma, not treat it. But understanding it is the first step.*
>
> *Check-in round:* ***How are you showing up today? What are you carrying into this room?****"*

Talking piece goes around.

Review & Reflection (10 minutes)

> **Facilitator:**
>
> *"Quick check on last week's homework.* ***Who continued the emotion tracking log? What patterns did you notice?****"*

Copyright © 2026 Positive Outreach Development Society, Inc.

Allow 3-4 responses. Common patterns people notice:

- Same emotions at the same time of day (evenings are often hardest)
- Certain people or situations are reliable triggers
- The same underlying emotion keeps showing up beneath different situations

> *"Good. Those patterns aren't random. They connect to your history — to experiences that taught your brain and body to respond in specific ways. That's exactly what we're going to unpack this week."*

Instruction: Understanding Trauma (40 minutes)

Part 1: What Trauma Actually Is (15 min)

> **Facilitator:**
>
> *"Let's start by getting clear on what trauma actually is — because the word gets used a lot and means different things to different people.*
>
> *SAMHSA — the Substance Abuse and Mental Health Services Administration — defines trauma using three E's: **Event, Experience, and Effect.** It takes all three for something to be trauma."*

Hand out SAMHSA Three E's handout.

> *"**Event.** Something happens. It could be a single incident — an assault, an accident, a loss. Or it could be ongoing — years of abuse, neglect, living in a violent environment, poverty, chaos in the home. The event is the thing that happened.*
>
> ***Experience.** How the person lived through it. Two people can go through the same event and experience it differently. A 5-year-old who witnesses violence experiences it differently than a 25-year-old. A child who has a supportive adult afterward experiences it differently than a child who has no one. The experience includes how frightened you were, how alone you were, how long it lasted, and how much control you had.*
>
> ***Effect.** The lasting impact. How the event and the experience change you — your brain, your body, your behavior, your relationships, your worldview. The effect is what you carry forward.*
>
> *Trauma isn't just what happened. It's what happened, how you lived through it, and what it did to you going forward. All three."*
>
> **Facilitator:**
>
> *"Here's something important. Trauma is not about being weak. Trauma is a normal human response to abnormal circumstances. If a child grows up in a home where the adults are violent and unpredictable, that child's brain adapts — it learns to scan for danger constantly, to react fast, to trust no one. Those adaptations make sense in that environment. They're survival strategies.*

Copyright © 2026 Positive Outreach Development Society, Inc.

The problem is that the brain doesn't automatically turn off those adaptations when the environment changes. You leave that home, but your brain is still scanning for danger. You're in a safe situation, but your body is still in survival mode. That's what trauma does — it keeps you responding to the past even when you're living in the present."

Part 2: ACEs — Adverse Childhood Experiences (15 min)

Facilitator:

"In 1998, two researchers — a doctor named Felitti and an epidemiologist named Anda — published the first results of a massive study. They surveyed over 17,000 people about hard things they experienced before the age of 18. They called these Adverse Childhood Experiences — ACEs. The questionnaire that came out of that work counts 10 of them. Here's the list:

Abuse:

1. *Physical abuse*
2. *Emotional abuse*
3. *Sexual abuse*

Neglect:

1. *Physical neglect*
2. *Emotional neglect*

Household Dysfunction:

1. *A parent with a substance abuse problem*
2. *A parent who was incarcerated*
3. *Witnessing domestic violence*
4. *A parent with a mental health condition*
5. *Parental separation or divorce*

Each one you experienced counts as 1 point. Your total is your ACE score.

Here's what they found, and this shook the medical world:

The higher the ACE score, the higher the risk for nearly every negative outcome you can name. Heart disease. Cancer. Depression. Substance abuse. Suicide attempts. Incarceration. Relationship problems. Early death.

*Someone with an ACE score of 4 or more is **7 times more likely** to be an alcoholic. **12 times more likely** to attempt suicide. **32 times more likely** to have learning or behavioral problems.*

This isn't opinion. This is science. What happens to you as a child physically changes your brain and body."

Facilitator:

"Now — here's where this connects to every person in this room.

In the general population, about 60% of people have at least 1 ACE. About 12% have 4 or more.

Copyright © 2026 Positive Outreach Development Society, Inc.

> ***Among incarcerated men, studies show that 75 to 90% have experienced significant trauma. The average ACE score is significantly higher than the general population.***
>
> *This doesn't mean everyone in here is a victim. It means that if you're in here, there's a high probability that something happened to you before you made the choices that got you here. And understanding that connection — between what happened TO you and what you DID — is critical."*

Part 3: Trauma Is Not an Excuse (10 min)

> **Facilitator:**
>
> *"I need to say this clearly, because if I don't, someone will use this material the wrong way.*
>
> ***Trauma is not an excuse.***
>
> *Understanding your trauma does not erase your accountability. 'I had a rough childhood' doesn't undo the harm you caused. 'My father was abusive' doesn't make your choices someone else's fault.*
>
> *But here's what trauma IS: **it's an explanation.** It explains how you got here — not to let you off the hook, but to show you the path forward.*
>
> *Think of it this way. If you're driving a car and you keep veering to the right, you need to know why. If the alignment is off, you can fix it. But you still caused the damage you caused while the car was pulling right. The alignment problem explains why you kept veering — it doesn't excuse the damage.*
>
> *Trauma explains the wiring. It explains why certain situations trigger you, why certain emotions overwhelm you, why certain patterns keep repeating. Once you understand the wiring, you can work with it. You can rewire.*
>
> *The man who says 'My trauma made me do it' is still in victim stance — still letting his past run his present. The man who says 'My trauma shaped me, but I'm learning to shape myself' — that man is free. Not free from pain. Free from being controlled by it.*
>
> *That's the difference. And that's what FORGE is building."*

Practice/Exercise: Anonymous ACE Score Activity (30 minutes)

> **Facilitator:**
>
> *"We're going to do an activity now. This is **completely anonymous.** No one will know your individual score — not me, not anyone.*
>
> *I'm going to hand out a questionnaire with the 10 ACE categories. All you do is read each one and check 'yes' or 'no' based on whether you experienced it before the age of 18. Then you count your total.*
>
> ***This paper is yours. You do not turn it in.** When you're done, fold it and put it in your journal. Or tear it up if you want. It's for your awareness, not mine."*

Hand out ACE questionnaires.

Copyright © 2026 Positive Outreach Development Society, Inc.

> *"Take about 5 minutes. Read each one carefully. Be honest with yourself. No one is watching."*

Give participants time to complete the questionnaire in silence. Facilitator should complete one too — modeling participation.

After 5 minutes:

> **Facilitator:**
>
> *"Everyone done? Good. Your score is between 0 and 10. Whatever it is, just hold it for a moment.*
>
> *Now — I want to do something with the group.* ***I'm not going to ask anyone to share their individual score.*** *But I want to get a picture of this room.*
>
> *I'm going to call out score ranges. If your score falls in that range, raise your hand. You can choose not to participate — that's fine.*
>
> *Score of 0 — raise your hand.*
>
> *Score of 1-2 — raise your hand.*
>
> *Score of 3-4 — raise your hand.*
>
> *Score of 5 or more — raise your hand."*

Look around the room. Let the group see the room.

> **Facilitator:**
>
> *"Look around. Look at how many hands went up at 3 or higher. At 5 or higher.*
>
> *This room is full of men who survived things as children that no child should have to survive. And you're still here. You're still standing. You're in a program designed to make you into someone who helps others.*
>
> *That doesn't erase the harm you've caused. But it does help explain the road that brought you here. And it tells you something else:* ***you are not alone in this.*** *Whatever you experienced, you are not the only man in this room who carries it.*
>
> *That matters. Because shame lives in secrecy. And healing starts when you realize that the thing you thought made you uniquely broken is actually shared by the men sitting next to you."*

Journal reflection (10 min):

> *"Take 10 minutes right now to write in your journal. The prompt is:* ***'How has my past shaped my present?'***
>
> *You don't have to write about specific events if you're not ready. You can write about patterns — patterns in your relationships, your reactions, your choices. How has what happened to you as a child shown up in who you are as an adult?*
>
> *This is private. No one reads it but you."*

Copyright © 2026 Positive Outreach Development Society, Inc.

Debrief & Discussion (20 minutes)

> **Facilitator:**
> *"I want to open the floor for reactions — not to the ACE score specifically, but to the whole concept.* ***What hits you about this material? What are you thinking or feeling right now?****"*

Allow open discussion. Common responses and how to handle them:

"I never thought of what happened to me as trauma."

> *"A lot of men don't. When something is all you've ever known — when everyone around you went through the same thing — it just feels normal. But 'normal' and 'healthy' aren't the same thing. What you experienced may have been normal for your environment. That doesn't mean it didn't affect you."*

"So you're saying it's not my fault I'm in here?"

> *"No. I'm saying two things at once. One: what happened to you as a child was not your fault. You didn't choose your parents, your neighborhood, or your circumstances. Two: the choices you made as an adult are your responsibility. Both things are true. Understanding your past doesn't erase your accountability — it gives you a map for change."*

"This feels like an excuse. Everybody had it hard."

> *"You're right that a lot of people had it hard. But 'everybody had it hard' doesn't mean everybody's pain is the same, and it doesn't mean pain doesn't affect you. If I break my arm and you break your leg, we both got hurt. We don't need to compare. We need to heal. Understanding trauma isn't about ranking who had it worse. It's about understanding what shaped you so you can reshape yourself."*

"I don't want to think about this."

> *"That's a valid response. And it might be cognitive indolence — we learned about that in Week 2. Not wanting to think about hard things is one of the most human reactions there is. But the hard things don't go away because we ignore them. They come out sideways — as anger, as addiction, as bad decisions. This week isn't about forcing you to relive your past. It's about understanding how it affects your present."*

Copyright © 2026 Positive Outreach Development Society, Inc.

Checkout & Assignment (10 minutes)

> **Facilitator:**
>
> *"Closing round. **One word that describes how you're feeling right now.** Just one word. No explanation needed."*

Talking piece goes around.

> **Facilitator (after the round):**
>
> *"Heavy session. I know. Here's your homework:*
>
> 1. ***Continue your journal entry on the prompt: 'How has my past shaped my present?'** Write at least a full page. Be honest. This is some of the most important writing you'll do in FORGE.*
> 2. ***Continue your emotion tracking log.** Pay attention this week to moments where your reaction seems bigger than the situation. Ask yourself: 'Is this about what just happened, or is this about something older?'*
> 3. ***Take care of yourself tonight.** This material can stir things up. Use your regulation techniques — 4-4-4 breathing, grounding. Talk to someone you trust if you need to. Don't sit alone with heavy feelings if they're pulling you under.*
>
> *Next session we're going to look at how trauma shows up in daily life in this facility — fight, flight, freeze, and fawn. You'll start recognizing these responses everywhere.*
>
> *See you next session."*

Copyright © 2026 Positive Outreach Development Society, Inc.

SESSION 11: Trauma Responses in the Dorm

Duration: 2 hours
Facilitator(s): Program Lead + Senior Mentor (if available)

Learning Objectives

By the end of this session, participants will be able to:

1. Name and describe the four trauma responses: fight, flight, freeze, and fawn
2. Identify how each trauma response looks in a correctional setting
3. Explain why certain dorm situations trigger disproportionate reactions
4. Distinguish between responding (conscious choice) and reacting (automatic trauma response)
5. Recognize trauma responses in themselves and others without judgment

Session 11 Checklist

- ☐ Elevator speech said together, then by 2–3 men alone
- ☐ Room set up in circle
- ☐ Trauma responses reference sheet distributed
- ☐ Opening circle completed — check-in on how participants are sitting with the material
- ☐ Review of Session 10 concepts
- ☐ Four trauma responses taught (fight, flight, freeze, fawn)
- ☐ Dorm-specific examples given for each response
- ☐ Triggers and disproportionate reactions explained
- ☐ Responding vs. reacting distinction made clear
- ☐ Small group exercise completed (3 dorm situations with trauma response analysis)
- ☐ Group share-out and discussion completed
- ☐ Default trauma response discussion held
- ☐ Closing round completed (personal primary response + situation)
- ☐ Homework assigned (observe trauma response + react vs. respond practice + review thinking reports)

Copyright © 2026 Positive Outreach Development Society, Inc.

Session Plan

Elevator Speech (2 minutes)

Start the session with the elevator speech, before anything else. The class says it together, out loud. Then ask two or three men to say it alone, without looking. Call on different men each session, so every man has said it alone before Session 18.

Opening Circle (10 minutes)

> **Facilitator:**
>
> *"Check-in. **How did you sleep last night? And how are you sitting with last session's material?***
>
> *Last session was heavy. Some of you may have had a hard night. That's normal. If stuff came up, name it. This room can hold it."*

Talking piece goes around.

Facilitator note: Pay close attention to anyone who seems significantly impacted. If someone says "I didn't sleep" or "I was up thinking all night," acknowledge it. If someone seems distressed beyond normal processing, plan a private check-in after the session.

Review & Reflection (10 minutes)

> **Facilitator:**
>
> *"Last session we learned that trauma is an Event, an Experience, and an Effect. We talked about ACEs. We talked about how trauma shapes your brain and body.*
>
> *One thing I want to reinforce: **understanding your trauma is not about looking backward and staying there.** It's about understanding why you respond the way you do so you can start choosing a different response.*
>
> *Today we get very practical. We're going to look at exactly how trauma plays out in the daily reality of this facility."*

Instruction: Fight, Flight, Freeze, and Fawn in Corrections (40 minutes)

Part 1: The Four Trauma Responses (15 min)

Copyright © 2026 Positive Outreach Development Society, Inc.

Facilitator:

"When your brain detects a threat — real or perceived — it activates your survival system. This is automatic. You don't choose it. It happens in milliseconds, before your thinking brain has a chance to weigh in.

There are four survival responses. You've probably heard of the first three. The fourth one is less well known but very common.

Fight. *Your brain says: 'The threat is here. Attack it. Overpower it. Destroy it before it destroys you.'*

What it looks like: aggression, hostility, yelling, posturing, violence, intimidation, controlling behavior. The person is oriented toward the threat and moving toward it.

Flight. *Your brain says: 'The threat is here. Run. Get away. Escape.'*

What it looks like: avoidance, withdrawal, changing the subject, leaving the room, staying busy to avoid thinking, substance use, sleeping all day. The person is oriented away from the threat.

Freeze. *Your brain says: 'The threat is here and I can't fight it or flee from it. Go still. Play dead. Disappear.'*

What it looks like: shutting down, going numb, staring blankly, not speaking, inability to make decisions, feeling paralyzed, dissociating — feeling like you're not really there, like you're watching from outside your body.

Fawn. *Your brain says: 'The threat is here. Appease it. Make it happy. If I give it what it wants, it won't hurt me.'*

What it looks like: people-pleasing, agreeing with everything, being overly nice or accommodating, abandoning your own needs to keep someone else calm, saying 'yes' when you mean 'no,' going along with things you know are wrong to avoid conflict."

Facilitator:

"Every one of these is a survival strategy. None of them are weaknesses. They developed because at some point in your life, they kept you alive or kept you safe. The kid who learned to fight back against an abusive parent — that was survival. The kid who learned to freeze and go invisible when the yelling started — that was survival. The kid who learned to please the violent adult to keep the peace — that was survival.

The problem isn't the response. The problem is when the response fires in situations where you're not actually in survival-level danger. And that happens constantly."

Part 2: What These Look Like in the Dorm (15 min)

Facilitator:

"Let's bring this into the dorm. Because this facility is full of trauma responses — we just don't call them that. We call them 'attitudes' or 'behavior problems' or 'that dude's crazy.' But most of the time, what we're seeing is a survival system that's firing when it doesn't need to.

Let me give you examples."

Copyright © 2026 Positive Outreach Development Society, Inc.

Fight response in the dorm:

> *"A CO tells a man to move from his bunk for a count. The man explodes. 'Don't tell me what to do!' He's in the CO's face. Everyone's thinking 'What's wrong with this guy? It's just a count.'*
>
> *But here's what's happening: that man's brain heard an authority figure giving a command, and it activated a fight response that was wired 20 years ago — by a father, or a foster parent, or a gang leader who used orders as a prelude to violence. His brain isn't responding to the CO. His brain is responding to every authority figure who ever hurt him. The CO is just the trigger."*

Flight response in the dorm:

> *"A man gets a letter from his lawyer with bad news about his case. Instead of talking about it, he sleeps for three days straight. Won't go to chow. Won't come to programming. Just disappears into his bunk. Everyone says 'He's being lazy' or 'He's given up.'*
>
> *What's actually happening: his system is in flight. The bad news activated a feeling of helplessness so intense that his brain said 'Get away. Escape.' And since he can't physically leave, his brain does the next best thing — it shuts him down. Sleep is flight when you can't run."*

Freeze response in the dorm:

> *"During a confrontation in the common area, a man just stands there. Doesn't speak. Doesn't defend himself. Doesn't move. Other guys start saying 'He's scared' or 'He's soft.'*
>
> *What's happening: his system froze. His brain detected a threat level that overwhelmed both the fight and flight options — so it went to the last resort. Freeze. This is especially common in men who experienced violence they couldn't escape as children. Their bodies learned that going still was the safest option. It's not cowardice. It's a wired response."*

Fawn response in the dorm:

> *"A man keeps giving away his commissary to someone who's pressuring him. He laughs off the pressure. He says it's no big deal. He's clearly uncomfortable but he keeps accommodating. People see him as weak or as a target.*
>
> *What's happening: his survival system is fawning. He learned somewhere — probably young — that the way to survive around dangerous people is to give them what they want. Make them happy. Don't resist. He's running a program that was installed when he was a child. In the dorm, it's making him a victim."*

Part 3: Why Situations Trigger Disproportionate Reactions (10 min)

Copyright © 2026 Positive Outreach Development Society, Inc.

> **Facilitator:**
>
> *"This is the key to understanding almost every conflict in this facility:*
>
> ***Most disproportionate reactions are not about the present situation. They are about a past wound being triggered by a present moment.***
>
> *When a man goes from zero to a hundred over someone bumping him in the hallway — that's not about the bump. Something about that moment activated an old survival response. Maybe being bumped felt like disrespect, and disrespect in his childhood meant danger. Maybe the physical contact triggered a body memory. Maybe the other person reminded him of someone who hurt him.*
>
> *The trigger is in the present. The reaction is from the past.*
>
> *This is what we mean by the difference between **responding** and **reacting.***
>
> ***Reacting** is automatic. It happens before thinking. It's your survival system running the show. It belongs to the past.*
>
> ***Responding** is conscious. It comes after the pause. It involves thinking, choosing, and deciding. It belongs to the present.*
>
> *Every tool we've taught you so far — the Think-Feel-Act cycle, the STOP technique, 4-4-4 breathing, grounding — is designed to create a gap between the trigger and your action. That gap is where you shift from reacting to responding. From past to present. From survival mode to chosen mode.*
>
> *As FORGE mentors, you'll see this every day. A man who's overreacting to something small. If you can look past the behavior and ask 'What was triggered?' — you'll be able to help him in a way that nobody else can."*

Practice/Exercise: Identifying Trauma Responses in Dorm Situations (30 minutes)

> **Facilitator:**
>
> *"We're going to work in groups of 3-4 for this exercise. Each group is going to identify 3 common dorm situations that trigger trauma responses.*
>
> *For each situation, I want you to discuss:*
>
> 1. ***What's the situation?** Describe it specifically.*
> 2. ***What trauma response does it typically trigger?** Fight, flight, freeze, or fawn?*
> 3. ***What might be underneath?** What past experience could this be connected to?*
> 4. ***What would a conscious response look like** — as opposed to the automatic reaction?*
>
> *You have 15 minutes in your groups."*

Facilitator circulates. Help groups think beyond the obvious. Push for the "what's underneath" question — that's where the real learning happens.

Copyright © 2026 Positive Outreach Development Society, Inc.

Share-out (15 min):

Each group presents one of their three situations to the full group.

> **After each presentation:**
> *"What else could be underneath that? What other past experiences might create that kind of reaction?"*

Allow brief group discussion after each. Build a shared understanding that most "behavior problems" are trauma responses.

Common situations groups might identify:

- Being told what to do by authority (triggers fight in men with abusive authority figures)
- Mail call with no mail (triggers flight/isolation in men with abandonment history)
- Sudden loud noise or commotion (triggers freeze in men with exposure to violence)
- Being pressured by a group (triggers fawn in men who learned appeasement for survival)
- Lockdowns with no information (triggers panic/freeze in men with experiences of helplessness)
- Someone standing too close (triggers fight in men with histories of physical/sexual abuse)
- Being accused of something you didn't do (triggers fight or flight in men who experienced injustice or false blame as children)

> **Facilitator (after share-out):**
> *"Look at this list. Look how many everyday situations in this facility can activate a trauma response. Now think about a dorm full of 50-60 men, most of whom have ACE scores of 4 or higher, all of them navigating these triggers every single day. That's the reality of this environment.*
> *And here's the hope: you are learning to see it. When you see it — in yourself and in others — you can respond differently. Instead of matching someone's fight energy, you can recognize it as a survival response and de-escalate. Instead of judging someone who freezes, you can understand what's happening and give them space. That awareness changes everything."*

Debrief & Discussion (20 minutes)

> **Facilitator:**
> *"I want to address something that might be coming up for some of you.* ***Which trauma response is yours?***
> *Not which one you think is best. Which one is your default — the one your system goes to automatically?"*

Allow open discussion. Many men will identify with fight. Some will be surprised to realize they also fawn or freeze in certain situations.

Copyright © 2026 Positive Outreach Development Society, Inc.

> **Facilitator:**
>
> *"Most people have a primary response and a secondary. And different situations can activate different responses. You might fight with peers but freeze with authority. You might fawn at home but fight in here. The responses aren't fixed — they're contextual.*
>
> *Here's what matters: **knowing your default gives you a chance to change it.** If you know your system goes to fight, you can intervene earlier. If you know you freeze under confrontation, you can prepare for those moments. Awareness is the first step to choice.*
>
> *One more thing. **Don't judge your response.** Whatever your brain learned to do, it learned for a reason. Fight isn't better than freeze. Fawn isn't weaker than flight. They're all survival strategies. The goal isn't to eliminate them — they might save your life someday. The goal is to make sure they're not running the show when you're not actually in danger."*

Checkout & Assignment (10 minutes)

> **Facilitator:**
>
> *"Closing round. **Name your primary trauma response — fight, flight, freeze, or fawn — and one situation this week where you saw it activate.**"*

Talking piece goes around. Facilitator goes first.

> **Facilitator (after the round):**
>
> *"Homework:*
>
> 1. ***Notice one trauma response this week — yours or someone else's.** Journal about it: What happened? What was the response? What do you think might have been triggered from the past?*
> 2. ***Practice the distinction between reacting and responding.** At least once before next session, catch yourself in a reaction and deliberately shift to a response. Use your tools — STOP, breathing, grounding. Journal what happened.*
> 3. ***Review your thinking reports from the last two weeks.** Look at them with new eyes. Were any of those situations actually trauma responses? Were any of your thinking errors actually your survival system firing?*
>
> *Next session we close out this week with Coping and Safety. We're going to build your personal toolkit for handling all of this — practically, realistically, in ways that actually work in this environment.*
>
> *See you next session."*

Copyright © 2026 Positive Outreach Development Society, Inc.

SESSION 12: Coping and Safety

Duration: 2 hours
Facilitator(s): Program Lead + Senior Mentor (if available)

Learning Objectives

By the end of this session, participants will be able to:

1. Distinguish between safe and unsafe coping strategies
2. Create a personal "Safe Coping Card" with 5 specific strategies
3. Explain FORGE's position: trauma-aware, not therapeutic
4. Identify when to refer someone to professional support
5. Articulate a personal safety plan for managing difficult emotions and situations

Session 12 Checklist

- ☐ Elevator speech said together, then by 2–3 men alone
- ☐ Room set up in circle
- ☐ Safe vs. Unsafe Coping handout distributed
- ☐ Blank Safe Coping Card templates distributed
- ☐ Opening circle completed — gratitude + struggle check-in
- ☐ Trauma response homework reviewed
- ☐ Safe vs. unsafe coping strategies taught with specific examples
- ☐ Self-care in correctional environment discussed realistically
- ☐ FORGE trauma-aware vs. therapeutic distinction clearly delivered
- ☐ Referral situations listed and explained
- ☐ Month's toolkit summarized (thinking errors → emotions → trauma → coping)
- ☐ Personal Safe Coping Cards created by all participants
- ☐ Share-out on Safe Coping Cards completed
- ☐ Four-week summary delivered
- ☐ Closing round completed
- ☐ Homework assigned (carry card + new coping strategy + tracking + reflection)

Copyright © 2026 Positive Outreach Development Society, Inc.

☐ Safety check: no participant left in visible distress

Session Plan

Elevator Speech (2 minutes)

Start the session with the elevator speech, before anything else. The class says it together, out loud. Then ask two or three men to say it alone, without looking. Call on different men each session, so every man has said it alone before Session 18.

Opening Circle (10 minutes)

> **Facilitator:**
> *"Check-in.* ***One thing you're grateful for today and one thing you're struggling with.*** *Both."*

Talking piece goes around.

Facilitator note: Gratitude paired with honesty. This check-in is designed to ground participants after two intense sessions while keeping them in honest territory.

Review & Reflection (10 minutes)

> **Facilitator:**
> *"We've covered a lot this week. Session 10: what trauma is, ACEs, and how your past shapes your present. Session 11: fight, flight, freeze, fawn — how your survival system shows up in the dorm.*
> ***Did anyone observe a trauma response this week — in yourself or someone else?****"*

Allow 2-3 responses. Affirm each observation:

> *"That's exactly the kind of awareness that makes FORGE different. Most people see behavior. You're starting to see what's driving the behavior."*
>
> **Facilitator:**
> *"Today we close out this week by getting practical. We know what trauma is. We know how it shows up. Now:* ***what do you do about it?*** *How do you cope — in a healthy way — in an environment that doesn't exactly make it easy?"*

Copyright © 2026 Positive Outreach Development Society, Inc.

Instruction: Safe vs. Unsafe Coping and FORGE's Boundaries (40 minutes)

Part 1: Safe vs. Unsafe Coping (15 min)

> **Facilitator:**
>
> *"Everyone copes. Every person in this room has strategies for dealing with stress, pain, and difficult emotions. The question isn't whether you cope — it's how.*
>
> *Coping strategies fall into two categories: **safe** and **unsafe.** The difference is simple: **safe coping helps you manage the pain without creating more problems. Unsafe coping manages the pain in the short term but creates bigger problems in the long term.**"*

Hand out Safe vs. Unsafe Coping Strategies sheet.

> *"Let's look at unsafe coping first — because I want you to recognize what you might already be doing."*

Unsafe Coping:

> *"**Substance use.** Drugs, alcohol, pruno, whatever's available. It numbs the pain temporarily. It also leads to addiction, health problems, disciplinary action, and more time.*
>
> ***Violence.** Using aggression to discharge emotional energy. It feels like release in the moment. It leads to harm — to others and to yourself.*
>
> ***Isolation and shutdown.** Withdrawing completely. Not eating. Sleeping all day. Refusing to engage. It feels like protection. It leads to depression and missed opportunities.*
>
> ***Risk-taking.** Gambling, debt, breaking rules for the thrill. It creates a distraction from pain. It creates new problems that compound the old ones.*
>
> ***Manipulation and control.** Trying to control everyone and everything around you to manage your own anxiety. It gives you an illusion of safety. It destroys relationships and trust.*
>
> ***Self-harm.** Hurting yourself to manage overwhelming emotional pain. If this is something you're doing, I need you to tell me — not for punishment, but for help. This is beyond what FORGE can address, and there are people who can help.*
>
> *Here's the thing about unsafe coping: **it works.** In the short term. That's why people keep doing it. The substance takes the edge off. The fight releases the pressure. The isolation stops the input. They work — until the consequences hit. And the consequences always hit."*
>
> *"Now — **safe coping.**"*

Copyright © 2026 Positive Outreach Development Society, Inc.

Safe Coping:

> *"**Physical activity.** Push-ups. Walking the yard. Any movement. Exercise burns off stress hormones — cortisol and adrenaline — naturally. Your body is designed for it.*
>
> ***Breathing and grounding.** You already know these. 4-4-4 breathing. 5-4-3-2-1 grounding. They work. Use them.*
>
> ***Journaling.** Writing about what you feel. Getting it out of your head and onto paper. You've been doing this for three weeks now. It's a coping strategy.*
>
> ***Talking to someone you trust.** Not everyone. Not randomly. But one person — a cohort member, a mentor, someone who can listen without judging.*
>
> ***Prayer or meditation.** Whatever your spiritual practice is. Many men in here find grounding through faith. That's real. Use it.*
>
> ***Creative expression.** Drawing, writing, poetry, music. If you have an outlet, use it. Creating something is the opposite of destroying something.*
>
> ***Routine and structure.** Having a predictable daily routine reduces anxiety. Wake up at the same time. Exercise at the same time. Write at the same time. Structure creates safety for a brain that's used to chaos.*
>
> ***Helping someone else.** Service Over Self isn't just a FORGE principle — it's a coping strategy. When you're focused on helping someone, you're not spiraling in your own pain. And you're doing something meaningful.*
>
> ***Naming the emotion.** Just saying 'I'm feeling grief right now' or 'This is shame' reduces the intensity. Research calls it 'affect labeling.' Putting a name on the feeling takes some of its power away."*
>
> **Facilitator:**
>
> *"Look at both lists. The safe list is available to every person in this room, right now, today. You don't need permission. You don't need a program. You don't need staff approval. Every single one of these is within your control.*
>
> ***The question isn't whether you have options. The question is whether you'll use them."***

Part 2: Self-Care in a Correctional Environment (10 min)

> **Facilitator:**
>
> *"I want to be real about something. 'Self-care' is a phrase that gets thrown around a lot, and in here, it can sound ridiculous. 'Take a bubble bath.' 'Light some candles.' 'Go to a spa day.' That's not your reality.*
>
> *Self-care in here is different. It's harder and it's more basic. Let me tell you what self-care actually looks like in this environment:*
>
> - *Drinking enough water instead of just coffee and Kool-Aid*
> - *Eating at chow even when you don't feel like it*
> - *Getting outside when yard is open — even for 10 minutes*
> - *Sleeping at night instead of staying up watching TV until 3 AM*

Copyright © 2026 Positive Outreach Development Society, Inc.

- *Brushing your teeth and keeping yourself clean — not because someone told you to, but because it's an act of self-respect*
- *Reading something that feeds your mind instead of numbing it*
- *Doing your FORGE homework — because working on yourself IS self-care*
- *Saying 'I'm not okay' to one person instead of pretending you're fine*

None of this is glamorous. All of it matters. The man who takes care of himself is better equipped to take care of others. You can't pour from an empty cup — even in here."

Part 3: FORGE Is Trauma-Aware, Not Therapeutic (10 min)

Facilitator:

"I need to draw a very clear line. This is important for you now and critical for when you become mentors.

FORGE is trauma-aware. FORGE is not therapy.

What does that mean?

Trauma-aware means: *We understand that most people in this facility have experienced significant trauma. We know how trauma affects thinking, emotions, and behavior. We create an environment where people can begin to understand their own history. We don't re-traumatize people. We don't force disclosures. We recognize trauma responses and respond with awareness instead of punishment.*

Trauma-aware does NOT mean: *We diagnose anyone. We treat PTSD or other conditions. We do trauma processing therapy. We ask people to relive their worst experiences in detail. We act as counselors, psychologists, or social workers.*

The line is this: ***FORGE helps you understand your trauma. FORGE does not treat your trauma.*** *If what you're carrying is bigger than what understanding can address — if you're having nightmares, flashbacks, suicidal thoughts, or symptoms that are interfering with your daily life — you need professional support. And there is no weakness in that.*

When you become a mentor, this line becomes even more important. Your mentee may tell you things that need professional help. Your job in that moment is to listen, to validate, and to refer. Not to fix. Not to treat. Not to play therapist. Listen, validate, refer.

I'm going to say it one more time because it matters: ***you are not a therapist. You are a peer mentor with lived experience who understands what these men are going through. That is enormously valuable. But it has limits. Know those limits and respect them.****"*

"Here are the situations where you always refer:

- *Someone talks about hurting themselves or ending their life*
- *Someone talks about hurting someone else*
- *Someone is experiencing flashbacks or severe dissociation*
- *Someone is in a mental health crisis that's beyond a bad day*
- *Someone discloses ongoing abuse (including sexual abuse in the facility)*

Copyright © 2026 Positive Outreach Development Society, Inc.

> *In these situations, you stay with the person. You keep them safe. And you get help. That's your job. That's enough."*

Part 4: When to Use What (5 min)

> **Facilitator:**
>
> *"Let me put the whole month together in terms of your toolkit:*
>
> ***When you notice a thinking error:*** *Use cognitive restructuring — challenge the thought, replace it.*
>
> ***When you're escalating emotionally:*** *Use 4-4-4 breathing or grounding to regulate.*
>
> ***When you need to make a decision under pressure:*** *Use the STOP technique.*
>
> ***When anger is covering something deeper:*** *Name what's underneath. Write about it. Talk to someone.*
>
> ***When a trauma response fires:*** *Recognize it. Pause. Remind yourself: 'This is my survival system. I'm not in danger right now. I have a choice.'*
>
> ***When the pain is too much to carry alone:*** *Use safe coping. Talk to a cohort member. Reach out for professional help if needed.*
>
> *You now have more tools than most people who've never been incarcerated. That's not an exaggeration. The average person walking down the street doesn't know what a thinking error is, can't name what they're feeling, and has no de-escalation tools. You do."*

Practice/Exercise: Creating Your Personal Safe Coping Card (30 minutes)

> **Facilitator:**
>
> *"Here's today's exercise. You're going to create a* ***Personal Safe Coping Card.*** *This is something you keep with you — in your pocket, in your journal, wherever you can reach it when you need it.*
>
> *On this card, I want 5 things:"*
>
> *"**1. My top 2 thinking errors** — the ones I fall into most. Write them down so you can catch them.*
>
> ***2. My primary trauma response** — fight, flight, freeze, or fawn. Know your default.*
>
> ***3. My body's early warning signal** — what does your body do when you're escalating? Jaw tightens? Chest gets hot? Stomach drops? Write down YOUR signal.*
>
> ***4. My 3 go-to safe coping strategies** — pick 3 from the list, or your own. Things you will actually do. Not aspirational — practical. Things available to you in this facility right now.*
>
> ***5. My person** — one person you can talk to when things get heavy. A cohort member, a mentor, a chaplain, anyone. Someone who will listen without judging. Write their name.*
>
> *This is your personal safety plan in wallet-sized form. When you're at Stage 2 on the escalation curve and climbing, you pull this out. You look at it. You use it."*

Give participants 15 minutes to create their cards.

Copyright © 2026 Positive Outreach Development Society, Inc.

Facilitator circulates. Help anyone who's stuck:

- "What coping strategy did you use this week that actually worked?"
- "Who in this room do you trust enough to talk to when things get hard?"
- "What's the first thing your body does when you start escalating? That's your signal."

Share-out (15 min):

> **Facilitator:**
>
> *"I want to go around the room. Share one thing from your card — any section. Just one thing."*

Allow each person to share one element. This normalizes the cards and builds mutual awareness.

> **Facilitator (after share-out):**
>
> *"Keep this card. It might look simple. It is simple. That's the point. In the middle of a crisis, simple is what works. You're not going to pull out a textbook. You're going to pull out this card. And it's going to remind you who you are and what you know."*

Debrief & Discussion (20 minutes)

> **Facilitator:**
>
> *"Let me pull all four weeks together. Because you've been on a journey, and I want you to see the map.*
>
> ***Week 1** — you committed. You said 'I'm in.' You learned what FORGE is and what it asks of you. You learned about Service Over Self.*
>
> ***Week 2** — you learned that your thinking controls your behavior. You identified your thinking errors. You learned to challenge them and replace them with better thoughts.*
>
> ***Week 3** — you learned to understand your emotions. You learned to regulate them instead of being controlled by them. You went beneath the anger and named what was really there.*
>
> ***Week 4** — this week — you learned about trauma. You understood how your past shapes your present. You learned to recognize survival responses. And today, you built a personal toolkit for coping safely.*
>
> *In four weeks, you've gone from walking in the door to having a full set of tools for understanding yourself — your thinking, your emotions, your history, and your responses. That's the foundation. That's Phase 1's 'Know Yourself' in action.*
>
> *We're one-third of the way through Phase 1. There's a lot more to come — communication, conflict resolution, accountability, empathy, problem-solving. But the hardest part — looking at yourself honestly — that's what you just did. Everything from here builds on this foundation."*

Copyright © 2026 Positive Outreach Development Society, Inc.

Open the floor:

> *"What's different about you now compared to four weeks ago? Not what you know — what's different about how you see yourself, how you move through the day, how you handle what comes at you?"*

Allow 4-5 responses. Don't force it if people need time to reflect. The changes may be subtle and hard to articulate — that's okay.

Checkout & Assignment (10 minutes)

> **Facilitator:**
>
> *"Closing round. **Complete this sentence: 'One thing I'm taking from this week that I will carry with me is...'**"*

Talking piece goes around. Facilitator goes last.

> **Facilitator (after the round):**
>
> *"Homework for next session:*
>
> 1. ***Carry your Safe Coping Card with you.** Use it at least once before next session. Journal what happened.*
> 2. ***Practice one new coping strategy** — something from the safe list that you haven't tried before. Push yourself slightly outside your comfort zone. Journal the result.*
> 3. ***Continue your emotion tracking log and thinking reports.** By now, these should be becoming habits. If they're not, recommit. The logs and reports are the evidence of your growth — and you'll need them for your Phase 1 portfolio.*
> 4. ***Write a reflection: 'What I understand now about myself that I didn't understand a month ago.'** At least half a page.*
>
> *Next week we move into Active Listening and Communication. We're shifting from internal work to relational work — how you interact with the people around you. You'll be surprised how much your communication changes when you understand your own thinking, emotions, and trauma responses.*
>
> *This was a hard week. I'm proud of this group. What you did this week — facing your own pain with honesty and courage — that's not common anywhere. Certainly not in here. Hold onto that.*
>
> *Take care of each other until next session. Service Over Self.*
>
> *See you next session."*

Copyright © 2026 Positive Outreach Development Society, Inc.